IN PATIENT HISTORY AND PHYSICAL EXAMINATION RECORD

Patient's Name _____ Age/Sex _____

UHID _____ IPD _____ D.O.A. 17/02/26

Room No. _____ Unit/Doctor _____

Time of Arrival 12:20pm Time of Assessment 12:20pm

Admitted From Emergency

Source of History Self

Relationship

Chief complaints
1 Sudden onset of severe gen-weakness c/o
2 Bodyache, inability walk by herself x 3-4 days
3 H/o blacklike stool x 2-3 days
4 H/o chest pain on ntm 16/2/26 (TROP-T negative at 1gm)
5

History of Present illness: Patient came c aforementioned c/o and admitted for further evaluation and management

Pain Scale

VAS Scale:

12 yrs. and above

0 1 2 3 4 5 6 7 8 9 10

None Mild Moderate Severe

Wong Baker Scale : 7-12 Yrs Age Group

0 NO HURT 2 HURTS LITTLEBIT 4 HURTS LITTLEMORE 6 HURTS EVENMORE 8 HURTS WHOLELOT 8 HURTS WORST

If pain score is >4 complete details

Duration: _____

Location: _____

Increasing factor (s) _____

Decreasing factors (s) _____

Chracteristics: Sharp/Dull/Aching/Burning/Radialing/Colic/Shooting (as appropriate)

Past History (If Yes, Please write details)

	Yes	No
Diabetes	✓	
Hypertension	✓	
Cardiac	✓	
Any other		

K/C/O CAD P.PTCA/HTN/DM2

PLAN OF CARE

Discharge planning

Social support needed at home	Yes	~~No~~	If Yes, Details
Home equipment enticipated	Yes	~~No~~	If Yes, Details
Physciotherapy anticipated at home	Yes	~~No~~	If Yes, Details
Wound care needs anticipated at home	Yes	~~No~~	If Yes, Details

Psychological assessment	Psychological referral needed	Yes	~~No~~
Functional needs	Physiotherapy referral	Yes	~~No~~
Nutritional needs	Dietary referrals	Yes	~~No~~
Restraint, if applicable	Physician/Chemical	Yes	~~No~~
Ay other referral		Yes	No

Family Education

Investigation advised	Treatment advised	Expected outcome/ Measurable goals
CBC/LFT/KFT/BLD GROUP/ECG CHEST XRAY (P/A) / STOOL FOR OCCULT BLOOD 2D ECHO (ASK) USG W/A HBsAg C CPK/CPK-MB IRON PROFILE (ASK)	Arrange 02 PRBC Inj MONOCEF 1g (ADM) Inj AZITHROC 500mg Inj OMESEE 40mg Inj TRANXIDA 1g (SOS) IV Inj LASIX 20mg (SOS) IV Inj MUCOSTA 100mg (ADM) in 0.9% NS (2ml/hr) T. IROSOMB (24hr/Hr)	(12hr/hr)

Declaration by the Patient/Relative/Attendant I hereby declare that the facts recorded above regarding my History are based on my narration and are accurate to the best of my knowledge	Name ..... Relationship with Patient ..... Date .....Time..... **Signature**
Name of Resident ASMH ID [redacted] Date & T[redacted] Signatur[redacted]	Counter Sign. By consultant in charge Name ASMH ID Date & Time Signature

[redacted] 71 Y/F

[redacted]

UHID P0005082 IPD No. 25/0927
DOA 17/Feb/2026 00:43

[redacted]

[redacted]

Name_____ Age/Sex_____ IPD No._____

UHID No._____ Diagnosis _____ Date _____

Notes

17/02/26 High Risk Consent

12:50am हमे हमारी भाषा मे बता दिया है कि हमारे मरीज कि हालत गंभीर है और उनका HB - 4.9 है, साथ ही ब्लड प्रेशर भी अधिक मात्रा मे बढ रहा है और घट रहा है। खून कम होने के वजह से और साथ ही ब्लड प्रेशर मे होते हुए बदलाव के वजह से जान का खतरा बना हुआ है, और यह सब जानते हुए हम अपने मरीज का ईलाज A.S.M.H मे करवाना चाहते है और यदि ऑपरेशन कोई भी अनहोनी होती है तो उसके जिम्मेदार हम खुद होंगे, इसमे अस्पताल या डॉक्टर का कोई दोष नही होगा।

नाम - [redacted]
रिश्ता - पति
दिनांक - 17-2-2026
हस्ताक्षर - [redacted]
समय - 1 A.M.

[redacted]

Mrs [redacted] 71 Y/F

[redacted] P0005082 IPD No. 25/0927

17/Feb/2026 00:43

[redacted]

[redacted]

Name _____ **Age/Sex** _____ **IPD No.** _____

UHID No. _____ **Diagnosis** _____ **Date** _____

Notes

17/02/2026
8:40 am.

C/S/B Dr Basu., RMO.

qc - stable.
c/o allergic reaction.
O/E - BP - 130/80 mm Hg
PR - 103/min
SpO2 - 98% + RA.
Afebrile.

Adv - RAT
Monitor vitals
Inj AVIL - 1 [redacted]

Noted by
Ombalam
17/2/26

[redacted] 71 Y/F

UHID P0005082 IPD No. 25/0927
DOA 17/Feb/2026 00:43

[redacted]

Name____ ge/Sex_____ IPD No._____

UHID No._____ Diagnosis _____ Date _____

Notes

17/2/26
9:30am

C/S/B Consulted

Dx: Severe Anemia
HTN CAD Post PTCA
T2DM [Newly diagnosed]

O/E BP - 130/70
PR - 107
SpO2 - 92%

Adv

- Transfuse 1 U PRBC
- T. Metformin 500 OD
- Rest CST
- Strict vitals monitoring
- Inform SOS to DMW

[redacted]

[redacted]

[redacted] 71 Y/F

[redacted]

UHID P0005082 IPD No. 25/0927

DOA 17/Feb/2026 00:43

[redacted]

[redacted]

Name_____ _Age/Sex_____ IPD No._____

UHID No._____ Diagnosis _____ Date _____

Notes

[redacted]

TRUST | CARE | CURE
(A Unit of A.S Kidney Hospital & Stone Center Pvt. Ltd.)

[redacted]

DOCTOR PROGRESS NOTES

Patient/Name Mrs. [redacted] 71 Y/F
W/o [redacted]
UHID P0005082 IPD No. 25/0927
DOA 17/Feb/2026 00:43
[redacted]

UHID
AGE
DATE

Please document Pain Score and action taken - if any. Please write all orders in order sheet.

Date & Time	Notes	Diet
17/02/2026. 8 PM	C/S/D Dr Basu, RMO.	
	qc- Improving	
	O/E BP - 130/65 mm Hg	
	PR - 90/min	
	SpO2 - 97% ↓ RA	
	Afebrile	
Note	Adv. RAT	[redacted]
	Monitor vitals	

Please document Pain Score and action taken - if any. Please write all orders in order sheet.

Date & Time	Notes	Diet
17/02/26. 8pm.	C/S/B Dr Bam, RMO.	
	GC - Improving	
	O/E BP - 126/76 mm Hg	
	PR - 72/min	
	SpO2 - 98% ↓ RA	
	Afebrile.	
	Adv. CAT	
	Monitor vitals	
	Physiotherapy.	
Noted by Sugandha 17/2/26; 8:20 pm		

[redacted]

Mrs [redacted] 71 Y/F

UHID P0005082 IPD No. 25/0927

DOA 17/Feb/2026 00:43

DR [redacted]

Name___ Age/Sex_____ IPD No._____

UHID No._____ Diagnosis _____ Date _____

Latest AHG gel Card Technology is used for Compatibility

Licence No 802B(H)

PACKED RED BLOOD CELLS I.P. (Vol)

100% Voluntary Blood Donations

Collected on	Expiry on	Issued on

Patient Age/Sex

Cr. No. Ward/Bed No.

Hospital / N. Home

This Unit is Non Reactive/ Negative for HIV I & II, HBsAg, HCV, VDRL & Malaria Parasite.

Donor No.

Instruction :

1. Store between = 2 to 6 degree C
2. Use sterile transfusion set with filter
3. Do not add any medication
4. Administer without warming
5. Do not infuse incase of visible deterioration
6. Properly identify intended reciplent
7. Bring Blood Bag to Room temperature

ABO Compatible

B

Rh Positive /

Tech. Sign.

BT NOTES.

To transfuse 1U PRBC slowly over. 3-4 hrs.

In case of BT related reaction.

BT Start time -

BT End time -

- Stop BT immediately
- Inj AVIL - 1amp - IV stat
- Inj HYDROCORT - 1amp IV/IM
- Inform DOD stat.

- Inj LASIX - 20mg - IV - after 2 hrs.

[redacted]

[redacted]

Mrs. [redacted] 71 Y/F
W/o [redacted]
UHID P0005082 IPD No. 25/0927
DOA 17/Feb/2026 00:43

[redacted]

Name_____ [redacted] ge/Sex_____ IPD No._____

UHID No._____ Diagnosis _____ Date _____

Notes

18/02/26.
8am.

A/S/B Dr Ban., RMO

GC- Improving

CPK MB - 12.49 (*). O/E BP - 132/78 mm Hg
PR - 76/min
SpO2 - 97% ↓ RA
Afebrile.

Adv. RAT [redacted]
Monitor vitals.
Physiotherapy.

Noted by Sugandha
18/2/26 ; 8:10am

[redacted]

Mrs. [redacted] 71 Y/F

W/o [redacted]

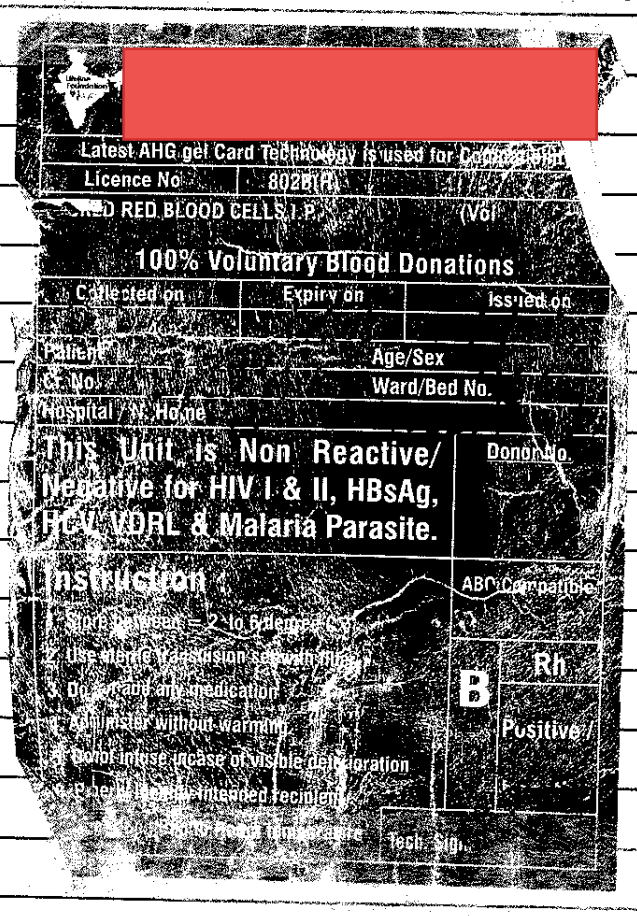
UHID P0005082 IPD No. 25/0927

DOA 17/Feb/2026 00:43

Name [redacted] Age/Sex _____ IPD No. _____

UHID No. _____ Diagnosis _____ Date _____

Notes


Latest AHG gel Card Technology is used for Compatibility

Licence No 802BTPA

RED RED BLOOD CELLS I.P. (Vol

100% Voluntary Blood Donations

Collected on	Expiry on	Issued on

Patient Age/Sex

C. No. Ward/Bed No.

Hospital / Home

This Unit is Non Reactive/ Negative for HIV I & II, HBsAg, HCV VDRL & Malaria Parasite.

Donor No.

Instruction

1. Store at Temp. = 2 to 6degree C
2. Use sterile transfusion set with filter
3. Do not add any medication
4. Administer without warming
5. Do not infuse incase of visible deterioration
6. Do not transfuse intended recipient

ABO Compatible

B

Rh

Positive /

Tech. Sign.

BT NOTES.

To transfuse 10 PRBC.

slowly over 4-5 hrs.

In case of any BT related reactions.

- stop BT immediately.
- Inj AVIL 1 amp i/v stat
- Inj HYDRO COR - 1 amp - i/v - stat
- call BOD. immediately.

- Inj LASIX - 20 mg - i/v after 2 hrs. from beginning of BT.

BT start time, 7:30 am.

BT end time -

[redacted]

Mrs [redacted] 71 Y/F

W/o [redacted]

UHID P0005082 IPD No. 25/0927

DOA 17/Feb/2026 00:43

[redacted]

[redacted]

HOSPITAL
WE CARE FOR YOU

Phone No: [redacted]

Name _____ Age/Sex _____ IPD No. _____

UHID No. _____ Diagnosis _____ Date _____

Notes

18/2/26
2 pm.

c/c/B Dr Bam, RMO

GC- Fair.

O/E BP – 130/76 mm Hg
PR – 76/min
SpO2 – 98% ↓ RA
Afebrile

2 @ PRBC transfused in total so far.

Adv. CAT
Monitor vitals
Syp Lasix 20 mL IV – stat [redacted]

Noted
18/2/26
2 pm
[illegible]

[redacted]

[redacted] 71 Y/F

W/o [redacted]

UHID P0005082 IPD No. 25/0927

DOA 17/Feb/2026 00:43

Name [redacted] Age/Sex _____ IPD No. _____

UHID No. _____ Diagnosis _____ Date _____

Notes

18/2/26

C/S/B Consultant

Dx/o Severe Anemia
HTN | CKD Post PTCA
T2DM

fever spikes (+)

O/E BP = 120/80
PR = 101
SpO2 - 95-96%

Adv
CBC
- Sr. Creatinine/BUN
- Gram profile
} 4m

Adv
- Transfuse 1 O PRBC
- Rest cst
- Strict vitals monitoring
- Inform SOS to DD

Ankit
Tehre
4:30pm

[redacted]

[redacted] 71 Y/F

D P0005082 IPD No. 25/0927

DOA 17/Feb/2026 00:43

Name [redacted] Age/Sex _____ IPD No. _____

UHID No. _____ Diagnosis _____ Date _____

Notes

19/02/2026
7:25 a.m.

C/S/B Dr Basu, RMO -

GC - Improving.

O/E BP - 132/74 mm Hg
PR - 71/min.
SpO2 - 97% ↓ RA
Afebrile.

Adv. - CST
Monitor vitals
CBC.
S. Folate / B 12
hom. Profile.

[redacted]

Noted by Sugandha
19/2/26 ; 7:30 a.m

[redacted]

W/O [redacted] 71 Y/F

UHID P0005082 IPD No. 25/0927

DOA 17/Feb/2026 00:43

Name [redacted] x IPD No.

UHID No. Diagnosis Date

Notes

19/2/26 C/S/B- RMO Dr. Arnab.

9:55-AM

(N)

GC=fever and impaired

No fresh complaints

2 PRBC given

O/E: CVS, CNS, RS, P/A — NAD.

Vitals: PR - 94/min, BP - 130/80 mmHg, SpO2 - 99% on RA

Adm: Monitor vitals, CSR as per chart, Refer SOS

C/S/B RMO Dr. Arnab 19/2/26

GC=fever and improving (N)

No fresh complaints

Rec IDA & b/l Serum Iron

b/l Transfusion

[redacted]

Noted by on Rounds 19/2/26

O/E:

Vitals: PR - 88/min, SpO2 - 98% RA, BP - 140/90 mmHg

CVS, CNS, RS, P/A — NAD

[redacted]

H C

Mr [redacted] 71 Y/F

W/ [redacted]

NUHID P0005082 IPD No. 25/0927

DOA 17/Feb/2026 00:43

U [redacted]

[redacted]

Age/Sex _____ IPD No. _____

Date _____

Notes

20/2/26

C/S/B Consultant

Dx/o Severe Anemia
HTN / CKD Post PTCA
T2DM (Newly diagnosed)

O/E BP = 130/80
PR = 82
SpO2 = 97%

— CBT

Pt willing for DOPR

— DOPR

[redacted]